

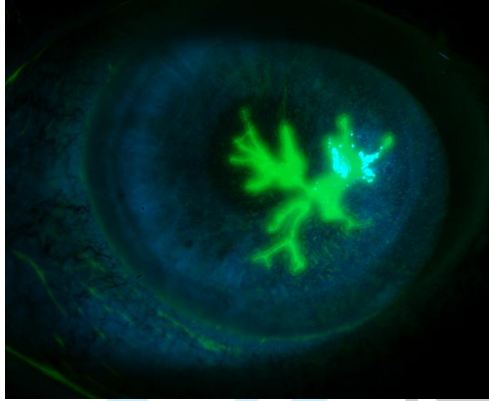


MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

QUESTIONS

Q1. A patient presents with painful red eye and the following findings on slit lamp examination. Which of the drugs will be contraindicated in this case?



- a) Acyclovir eye ointment
- b) Atropine
- c) Lubricating eye drops
- d) Topical steroids

Q2. A patient is diagnosed with Lisch nodules on slit lamp examination. Which of the following tumor is likely to be associated with this condition?

- a) Cavernous hemangioma
- b) Optic nerve glioma
- c) Retinoblastoma
- d) Lymphoma

Q3. A middle-aged lady has complaints of frequent headaches and menstrual irregularities. Her visual acuity is normal, but she is found to have loss of temporal fields in both eyes on confrontation field test in eye OPD. All of the following would be helpful in confirming the diagnosis in this case except:

- a) Humphrey's Perimetry
- b) Prolactin levels
- c) MRI brain
- d) USG Bscan orbit

Q4. Which of the following is not true about the fungal corneal ulcer?

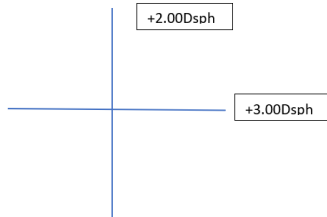
- a) Satellite lesions are seen.
- b) Corneal perforation is common.
- c) Trauma with vegetative trauma is a common risk factor.
- d) Ulcer margins are often indistinct.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- Q5. A young girl underwent retinoscopy under atropine done at 1 m working distance as shown. What will be the glass prescription for her?



- a) +1.0 Dcyl x 90 deg
- b) +2 Dsph/+1.0 Dcyl x 90 deg
- c) +3.0 Dcyl x 180 deg
- d) +1.0 Dcyl x 180 deg

- Q6. A 6 years old girl is brought to the emergency with history of high-grade fever. Her left is very painful with periorbital swelling with bulging of eyeball. On examination, the movements in the eye are restricted and there is afferent pupillary defect. There is also noted recent history of sinus infection. Which of the following can be possible diagnosis?

- a) Orbital cellulitis
- b) Preseptal cellulitis
- c) Sty
- d) Blepharitis

- Q7. A patient presents with small pupils which do not constrict in response to torchlight, but constrict when patient looks at a pen held near his face. Which of the following can be likely site of lesion in this condition?

- a) Pretectal nucleus in midbrain
- b) Ciliary ganglion
- c) Cervical Sympathetic chain
- d) Short Ciliary nerves

- Q8. A patient with known coronary artery disease develops sudden painless loss of vision in left eye. On examination, visual acuity is counting fingers and relative afferent pupillary defect is present. Fundus examination showed the following examination with a cherry red spot. What is the diagnosis in this case?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

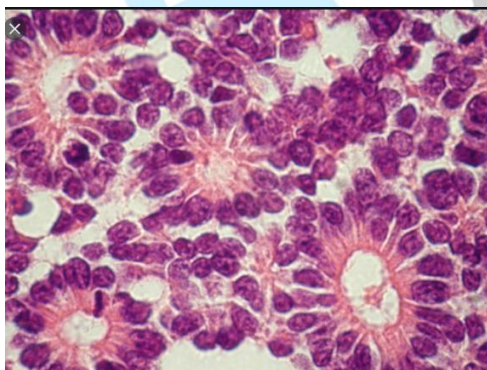


- a) Branch retinal artery occlusion
- b) Branch retinal vein occlusion
- c) Central retinal artery occlusion
- d) Central retinal vein occlusion

Q9. A patient who was started on antitubercular treatment for last 1 month, now complains of decreased vision. On examination, there is noted defective color vision and centrocaecal scotoma on perimetry. Which of the following drugs is implicated as the cause of this condition?

- a) Rifampicin
- b) Ethambutol
- c) Pyrazinamide
- d) Linezolid

Q10. An enucleation was done for a child presenting with intraocular mass filling more than half of his eyeball with calcification. Histopathological examination revealed the following findings. What is the diagnosis in this case?



- a) Rhabdomyosarcoma
- b) Dermoid
- c) Retinoblastoma



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

d) Melanoma

Q11. A mother brings her young child with drooping of upper eyelid in left eye which decreases whenever child eats or sips on the liquids. What is the likely diagnosis in this case?

- a) Congenital third nerve palsy
- b) Marcus Gunn Synkinetic ptosis
- c) Myasthenia gravis
- d) Facial nerve palsy

Q12. A young man got penetrating injury in his right eye by an iron nail. 2 weeks later, he developed painful red eye and decreased vision in left eye as well. What is the likely diagnosis in this case?

- a) Left acute anterior uveitis
- b) Post-traumatic ophthalmritis
- c) Siderosis bulbi
- d) Sympathetic ophthalmitis

Q13. All of the following are features of moderate diabetic retinopathy except?

- a) Cotton wool spots
- b) Dot hemorrhages
- c) Iris neovascularization
- d) Microaneurysms

Q14. Which of the following drug will be avoided to control high intraocular pressure in a case of acute anterior uveitis?

- a) Latanoprost
- b) Timolol
- c) Acetazolamide
- d) Brimonidine

Q15. The instrument shown in the image is used for:





MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026



- a) For Probing in congenital NLD obstruction
- b) For assisting in corneoscleral suturing
- c) Chalazion curettage
- d) To make corneoscleral tunnel

Q16. A 45-year-old woman has chronic GERD. Endoscopic biopsy from salmon-colored mucosa above the GE junction shows intestinal metaplasia without dysplasia. Best management?

- a) Esophagectomy
- b) PPI therapy with surveillance endoscopy
- c) Immediate chemoradiotherapy
- d) No follow-up required

Q17. A 35-year-old man with recurrent biliary colic and confirmed cholecystolithiasis requests laparoscopic cholecystectomy. During consenting, you mention the risk of common bile duct injury. The overall incidence of CBD injury in laparoscopic cholecystectomy is:

- a) <0.1%
- b) 0.3–0.5%
- c) 1–2%
- d) 5%

Q18. A 45-year-old man undergoes laparoscopic cholecystectomy. The cystic duct is wider than expected (10mm). Intraoperative cholangiogram (IOC) shows multiple filling defects in the CBD. What is the MOST appropriate next step?

- a) Complete cholecystectomy and arrange ERCP post-operatively



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

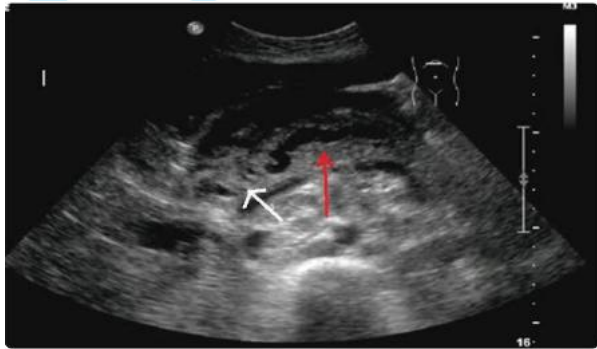
MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- b) Convert to open surgery and perform common bile duct exploration
- c) Laparoscopic common bile duct exploration (LCBDE) if skill available, OR post-operative ERCP
- d) Place a T-tube in the cystic duct and close

Q19. A 25-year-old woman is found to have a solitary 2 cm cystic lesion in the liver on USS. MRCP shows direct communication with the intrahepatic biliary system. She has lived in a sheep-farming area. Which investigation MOST confirms the diagnosis?

- a) Serum AMA antibody — diagnostic for PBC
- b) Serum echinococcal antibody (indirect haemagglutination) + USS showing 'daughter cysts' — hydatid disease
- c) Serum AFP — confirms hepatocellular carcinoma
- d) Serum CA 19-9 — confirms cholangiocarcinoma

Q20. An MRCP shows dilatation of both the common bile duct (15mm) and the pancreatic duct (8mm) — the 'double duct sign.' In a 65-year-old man with painless jaundice and weight loss, what is the MOST likely diagnosis?



- a) Choledocholithiasis
- b) Primary sclerosing cholangitis
- c) Carcinoma of the head of pancreas
- d) Chronic pancreatitis

Q21. A 70-year-old woman presents with colicky pain and absolute constipation. AXR shows a large inverted U-shaped loop in the left iliac fossa/left flank — the 'coffee bean' sign. She is haemodynamically stable. What is the diagnosis and first-line management?

- a) Large bowel obstruction from carcinoma — emergency Hartmann's procedure
- b) Sigmoid volvulus — rigid sigmoidoscopy/flexible sigmoidoscopy decompression as first-line (80% success); elective sigmoid colectomy after few weeks



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- c) Ogilvie's syndrome — neostigmine IV
- d) Caecal volvulus — right hemicolectomy

Q22. A 48-year-old man presents with a 4mm stone in the right ureter at the level of the iliac vessels (mid-ureter) causing moderate hydronephrosis. He is afebrile, haemodynamically stable, and pain is controlled. He is a Type 1 diabetic on insulin. eGFR 55 mL/min (reduced). What modification to standard medical expulsive therapy is REQUIRED?

- a) No modification — standard tamsulosin + NSAIDs are appropriate
- b) Avoid NSAIDs — use paracetamol ± opioids instead; NSAIDs contraindicated in CKD (eGFR <60) and diabetes due to risk of acute-on-chronic kidney injury
- c) Use of corticosteroids — reduces ureteric oedema
- d) Immediate ureteroscopy — CKD is an absolute contraindication to MET

Q23. A 60-year-old man post-partial gastrectomy is in 2L positive fluid balance on day 2. Which is the MOST dangerous consequence of cumulative positive fluid balance in GI surgery?

- a) Dilutional hyponatraemia
- b) Anastomotic leak due to bowel wall oedema and reduced perfusion
- c) Hyperchloraemic metabolic acidosis
- d) Worsening chronic kidney disease

Q24. A 19-year-old man undergoes emergency laparoscopic appendicectomy for acute appendicitis. Intraoperatively, the appendix appears grossly normal. The rest of the abdomen is explored — terminal ileum shows 60cm of thickened, hyperaemic bowel with fat wrapping and creeping of the mesentery. Mesenteric lymph nodes are enlarged. The caecum is uninvolved. What is the MOST likely finding and intraoperative decision?

- a) Caecal carcinoma — proceed with right hemicolectomy
- b) Mesenteric adenitis — close abdomen, appendix removal unnecessary
- c) Crohn's disease of terminal ileum — close abdomen without appendicectomy if the caecum is uninvolved; refer to gastroenterology
- d) Yersinia enterocolitica infection — appendicectomy anyway + antibiotics

Q25. A 45-year-old woman is involved in a road traffic accident with a pelvic fracture. She has blood at the vaginal introitus and urethral meatus. She is unable to void. CT pelvis shows pelvic fracture with a urethral injury at the bladder neck. Which statement regarding female urethral trauma is CORRECT?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- a) Female urethral injuries are more common than male — shorter urethra is more vulnerable
- b) Female urethral injuries are rare (incidence 0–6% of pelvic fractures in females vs 10–25% in males) due to the shorter and more mobile female urethra; bladder neck injuries are more common in females
- c) Management is identical to males — suprapubic catheter + delayed urethroplasty
- d) Female urethral injuries always require immediate hysterectomy

Q26. A 36-year-old woman presents with:

- Recurrent renal stones
- Bone pain
- Constipation

Labs:

- Calcium 13.5 mg/dL
- Low phosphate
- Elevated PTH

Most likely diagnosis?

- a) Secondary hyperparathyroidism
- b) Primary hyperparathyroidism
- c) Hypoparathyroidism
- d) Vitamin D deficiency

Q27. A 55-year-old woman presents with:

- Bloody nipple discharge
- No palpable lump
- Mammography normal

Ductography reveals a filling defect within a major lactiferous duct.

Most likely diagnosis?

- a) Fibroadenoma
- b) Intraductal papilloma
- c) Phyllodes tumor
- d) Fat necrosis



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q28. A 48-year-old diabetic presents with a postoperative wound infection.

Histology from wound edge demonstrates:

- Numerous fibroblasts
- New capillary formation
- Loose extracellular matrix

Which phase of wound healing is represented?

- a) Hemostasis
- b) Inflammatory phase
- c) Proliferative phase
- d) Maturation phase

Q29. A malnourished oesophageal cancer patient with dysphagia needs 2 weeks pre-op nutritional support. Best route?

- a) TPN via central line for 2 weeks
- b) Nasojejunal tube feeding for 2 weeks
- c) High-calorie oral supplements alone
- d) Nasogastric tube feeding

Q30. A 46-year-old man presents with progressive dysphagia for 8 months. Initially he had difficulty swallowing solids, but over the last 2 months he has also developed dysphagia to liquids. He has lost 12 kg weight and complains of retrosternal discomfort. Upper GI endoscopy reveals an ulceroproliferative lesion in the middle third of the esophagus.

Which of the following is the MOST IMPORTANT prognostic factor in this patient?

- a) Histological grade of tumor
- b) Tumor length on endoscopy
- c) Presence of lymph node metastasis
- d) Presence of dysphagia

Q31. A 58-year-old chronic smoker presents with progressive dysphagia.

Upper GI endoscopy:

- Lower-third esophageal growth



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Biopsy:

- Adenocarcinoma

PET-CT:

- Celiac lymph node involvement

No liver metastasis.

According to TNM classification, celiac node involvement is best classified as:

- a) Local nodal disease
- b) Regional nodal disease
- c) Distant metastatic disease
- d) Peritoneal metastasis

Q32. A 40-year-old woman presents with recurrent hypoglycemic episodes. Biochemistry confirms endogenous hyperinsulinism. CT and MRI fail to localize the lesion. Endoscopic ultrasound suggests a 1.2-cm lesion in pancreatic body. No MEN features.

Best treatment?

- a) Enucleation of tumor
- b) Whipple procedure
- c) Total pancreatectomy
- d) Chemotherapy alone

Q33. A 70-year-old woman has a 2 cm ampullary tumour found at ERCP performed for obstructive jaundice. Biopsy confirms ampullary adenocarcinoma. EUS shows T2N0 disease (muscularis propria invasion, no nodes). What is the MOST appropriate treatment?

- a) Endoscopic ampullectomy — sufficient for T2 disease
- b) Pancreaticoduodenectomy (Whipple's procedure) — standard curative resection for T2+ ampullary carcinoma
- c) Gemcitabine + cisplatin chemotherapy alone
- d) ERCP stenting for palliation

Q34. A 28-year-old man presents with a painless right testicular swelling. USS shows a 3 cm hypoechoic intratesticular mass. Pre-orchidectomy tumour markers are: AFP 1240 ng/mL (elevated, normal <10), β -



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

hCG 45 IU/L (mildly elevated, normal <5), LDH 280 U/L (mildly elevated). He undergoes right radical orchiectomy via an inguinal approach. Histology shows a mixed germ cell tumour — 60% embryonal carcinoma, 30% yolk sac tumour, 10% teratoma. Post-orchidectomy markers at 4 weeks: AFP 920 ng/mL (still elevated — inadequate decline), β -hCG 3 IU/L (normalised). CT chest/abdomen/pelvis shows no retroperitoneal or distant metastases. What does the persistently elevated AFP indicate and what is its HALF-LIFE significance?

- a) AFP elevation confirms seminoma — no further significance
- b) Persistently elevated AFP after orchiectomy suggests residual disease or metastases despite negative CT — AFP half-life is 5–7 days; inadequate decline indicates occult metastatic disease
- c) AFP is non-specific — its elevation is irrelevant to management
- d) Elevated AFP is expected for 6 months post-orchidectomy — normal biological clearance

Q35. A 45-year-old woman presents with:

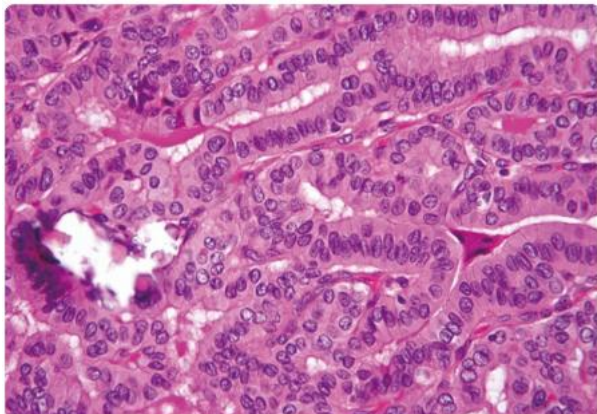
- Weight loss
- Neck swelling

Examination:

- Solitary thyroid nodule
- Hard
- Irregular
- Cervical lymphadenopathy

FNAC:

- Nuclear grooves
- Orphan Annie nuclei



Diagnosis?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

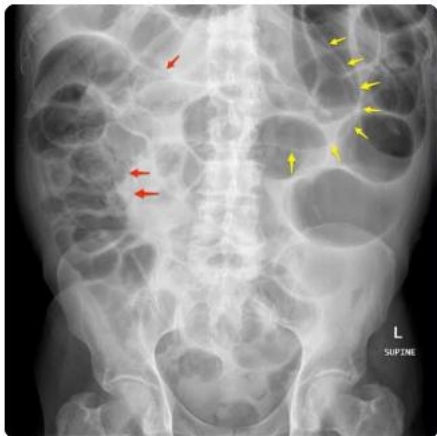
MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- a) Follicular carcinoma
- b) Papillary carcinoma
- c) Medullary carcinoma
- d) Anaplastic carcinoma

Q36. A 45-year-old man presents with a 4 cm midline neck swelling that appears immediately below the hyoid. It is soft, fluctuant, transilluminable, and moves on tongue protrusion. FNAC shows amylase-rich fluid with thyroid follicular cells. What is the management?

- a) Simple aspiration — thyroglossal cysts resolve after aspiration
- b) Sistrunk's operation — complete excision of cyst + central body of hyoid + thyroglossal tract to foramen caecum
- c) Total thyroidectomy — ectopic thyroid tissue must be removed
- d) Sclerotherapy with alcohol injection

Q37. A 10-day-old male neonate born at 30 weeks develops abdominal distension, bloody stools, and bilious aspirates. AXR shows pneumatosis intestinalis and portal venous gas. He deteriorates with worsening acidosis (pH 7.18) and thrombocytopenia. What is the ABSOLUTE surgical indication in NEC?



- a) Portal venous gas alone
- b) Pneumatosis intestinalis alone
- c) Pneumoperitoneum (free air on AXR) — confirming intestinal perforation
- d) Bloody stools + distension

Q38. A 60-year-old hypertensive man collapses at home. His wife says he complained of "the worst headache of his life" immediately before becoming unresponsive. On arrival to ED: GCS 7 (E2V2M3).



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

CT head shows subarachnoid haemorrhage (Fisher Grade 4 — diffuse SAH with intraventricular extension). His GCS deteriorates to 5 over the next 20 minutes. Which of the following CORRECTLY uses the GCS in the context of SAH management?

- a) GCS 5 in SAH indicates World Federation of Neurosurgical Societies (WFNS) Grade V — associated with poorest prognosis; intubation + emergency neurosurgical referral
- b) GCS deterioration in SAH is a normal finding — observe and reassess in 6 hours
- c) GCS 5 is equivalent to Hunt and Hess Grade I — good prognosis
- d) GCS is not used in SAH grading — only CT findings determine management

Q39. A trauma patient presents with:

- Hypotension
- Distended neck veins
- Muffled heart sounds

Diagnosis?

- a) Hemothorax
- b) Cardiac tamponade
- c) Pneumonia
- d) Flail chest

Q40. 30 minutes into transfusion: fever 39.2°C, rigors, back pain, haemoglobinuria (port-wine urine), hypotension. IMMEDIATE management?

- a) Give paracetamol and slow transfusion rate
- b) STOP transfusion, IV 0.9% NaCl, send blood + unit to lab, catheterise, manage DIC
- c) IV adrenaline 0.5 mg IM + antihistamine + hydrocortisone
- d) Broad-spectrum IV antibiotics and continue transfusion

Q41. A 30-year-old woman with known IgA deficiency requires a blood transfusion. Which special blood product is required?

- a) CMV-negative blood
- b) Irradiated blood
- c) IgA-depleted / washed RBCs
- d) Leucodepleted blood only



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q42. Transfusion-Related Acute Lung Injury (TRALI) — which statement is CORRECT?

- a) Occurs >6 hours after transfusion; caused by fluid overload
- b) Caused by donor anti-HLA/anti-HNA antibodies activating recipient neutrophils; onset within 6 hours
- c) Treated with diuretics and fluid restriction
- d) Only occurs with pRBC transfusion

Q43. A 50-year-old woman undergoes emergency colectomy for fulminant UC. Intraoperatively she receives 12 units pRBC. 6 hours post-op she develops fever, rigors, and haemolytic anaemia. Direct Coombs test is positive. What is the diagnosis?

- a) Febrile non-haemolytic transfusion reaction
- b) Delayed haemolytic transfusion reaction (DHTR)
- c) Acute haemolytic transfusion reaction
- d) Transfusion-associated sepsis

Q44. A 28-year-old woman is brought after a road traffic accident. She has GCS of 7 (E1V2M4), bilateral fixed dilated pupils, and Cushing's triad (hypertension + bradycardia + irregular breathing). CT head shows a large epidural haematoma (60 mL, 20mm thick, 10mm midline shift) in the temporal region with uncal herniation. What is the IMMEDIATE management?

- a) IV mannitol 1g/kg + hyperventilate to PaCO₂ 30–35 mmHg + IV dexamethasone + urgent neurosurgical evacuation
- b) IV dexamethasone 10 mg immediately
- c) ICP monitor insertion only
- d) Repeat CT head in 4 hours

Q45. A 35-year-old man is stabbed in the left chest. On arrival:

- BP: 78/46 mmHg
- Pulse: 138/min
- Respiratory rate: 34/min
- Neck veins are distended
- Heart sounds are muffled
- Breath sounds are equal bilaterally
- FAST shows pericardial fluid

Most likely diagnosis?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- a) Massive hemothorax
- b) Tension pneumothorax
- c) Cardiac tamponade
- d) Neurogenic shock

Q46. A 6-year-old girl has breast budding and vaginal bleeding. Ovarian mass biopsy shows Call-Exner bodies. Diagnosis?

- a) Sertoli-Leydig cell tumor
- b) Granulosa cell tumor
- c) Fibroma
- d) Dysgerminoma

Q47. A 24-year-old woman with primary amenorrhea and elevated LDH develops an abdominal mass. Diagnosis?

- a) Yolk sac tumor
- b) Sertoli-Leydig tumor
- c) Dysgerminoma
- d) Immature teratoma

Q48. Which of the following is true regarding a succenturiate lobe of placenta?

- a) It is a type of velamentous cord insertion
- b) Vessels are protected by Wharton's jelly
- c) It is associated with retained placenta
- d) Umbilical cord inserts into the accessory lobe

Q49. Narrowest diameter of fetal skull in vertex presentation that passes through pelvis is:

- a) Suboccipitofrontal
- b) Suboccipitobregmatic
- c) Occipitofrontal
- d) Mentovertebral

Q50. At what gestational age does the uterine fundus reach the umbilicus?

- a) 12 weeks
- b) 20 weeks
- c) 24 weeks
- d) 28 weeks



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- Q51. Which of the following is true about partial molar pregnancy?
- a) Contains no fetal parts
 - b) Most commonly 46XX
 - c) Trophoblastic proliferation is diffuse
 - d) Karyotype is usually triploid
- Q52. A woman with 6 weeks amenorrhea, abdominal pain, vaginal bleeding, β -hCG 2200 mIU/mL, no intrauterine sac. Most likely diagnosis?
- a) Missed abortion
 - b) Early viable intrauterine pregnancy
 - c) Complete mole
 - d) Ectopic pregnancy
- Q53. Best describes the alert line in the partogram?
- a) Indicates maternal distress
 - b) Represents normal labor progress (1 cm/hr cervical dilatation)
 - c) Indicates time to start augmentation
 - d) Signals immediate cesarean section
- Q54. Which structure protects the umbilical cord vessels?
- a) Wharton's jelly
 - b) Amniotic membrane
 - c) Allantoic duct
 - d) Yolk sac remnant
- Q55. Fitz-Hugh-Curtis syndrome, adhesions are noted in:
- a) Omental adhesions
 - b) Tubal region
 - c) Perihepatic region
 - d) Intrauterine adhesions
- Q56. Tandem radiotherapy is used in:
- a) Cervical cancer
 - b) Ovarian cancer
 - c) Endometrial cancer



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

d) Choriocarcinoma

Q57. A patient presents with 45 days amenorrhea and wants MTP. Appropriate regimen is:

- a) Mifepristone 200 mcg & Misoprostol 800 mg after 48 hours
- b) Misoprostol 200 mg & Methotrexate after 48 hours
- c) Mifepristone 200 mg & Misoprostol 800 mcg after 48 hours
- d) Misoprostol 800 mcg & Mifepristone 200 mg after 48 hours

Q58. Combined oral contraceptive pills reduce all, EXCEPT:

- a) Simple ovarian cyst
- b) Menorrhagia
- c) Premenstrual syndrome
- d) Hepatic adenoma

Q59. A 15-year-old girl presents with cyclical abdominal pain, urinary retention & primary amenorrhea. Likely diagnosis:

- a) Transverse vaginal septum
- b) Longitudinal vaginal septum
- c) Imperforate hymen
- d) Septate uterus

Q60. A 16-year-old girl presents with primary amenorrhea. Breasts well developed, no axillary/pubertic hair. Lump in inguinal region. USG: absent uterus. Likely diagnosis:

- a) Androgen insensitivity syndrome
- b) MRKH syndrome
- c) Turner syndrome
- d) Kallmann syndrome

Q61. Hysteroscopy is indicated in all of the following, EXCEPT:

- a) Asherman syndrome
- b) Menorrhagia
- c) Infertility
- d) Adenomyosis

Q62. A 28-year-old woman presents with continuous urinary leakage through the vagina, 4 months after prolonged obstructed labor. Most likely fistula?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- a) Urethrovaginal fistula
- b) Ureterovaginal fistula
- c) Vesicovaginal fistula
- d) Rectovaginal fistula

Q63. A 32-year-old woman presents with secondary amenorrhea, pelvic pain, intermittent fever, weight loss, and adnexal mass. Most likely diagnosis?

- a) PCOS
- b) Endometriosis
- c) Genital tuberculosis
- d) Ectopic pregnancy

Q64. Glands of Cloquet are involved in:

- a) Lymphatic drainage of vulva
- b) Lymphatic drainage of cervix
- c) Lymphatic drainage of uterus
- d) Lubricating glands of vagina

Q65. A 30-year-old woman at 32 weeks gestation presents with painless vaginal bleeding and sinusoidal fetal heart rate pattern. Most appropriate test to determine the source of bleeding?

- a) Kleihauer-Betke test
- b) Doppler ultrasound
- c) Fetal fibronectin test
- d) Apt test (Alkaline denaturation test)

Q66. Match the following signs with their descriptions:

1. Chadwick's sign	a. Increased pulsations felt through lateral fornices
2. Goodell's sign	b. Softening of cervix.
3. Oslander's sign	c. Bluish hue of vaginal wall.
4. Hegar's sign	d. Abdominal and vaginal fingers oppose below the body of uterus.

- a) 1-a, 2-d, 3-c, 4-d
- b) 1-c, 2-a, 3-b, 4-d
- c) 1-c, 2-b, 3-a, 4-d
- d) 1-b, 2-c, 3-d, 4-a

Q67. Spiegelberg's criteria are used to diagnose:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- a) Ovarian pregnancy
- b) Cesarean scar pregnancy
- c) Abdominal pregnancy
- d) Cervical pregnancy

Q68. A woman develops persistent vaginal bleeding and elevated β -hCG after a molar pregnancy. Most common site of metastasis?

- a) Bone
- b) Kidney
- c) Liver
- d) Lungs

Q69. What is the minimum number of antenatal visits recommended by WHO for a healthy pregnancy?

- a) 4
- b) 8
- c) 10
- d) 12

Q70. A primigravida undergoes episiotomy due to shoulder dystocia. Which structure is typically NOT cut?

- a) Posterior vaginal wall
- b) Bulbospongiosus
- c) Levator ani
- d) Ischiocavernosus

Q71. A 32-year-old woman at 34 weeks gestation presents with sudden vaginal bleeding, severe abdominal pain, tender abdomen, and fetal distress. Most likely diagnosis?

- a) Placenta previa
- b) Abruptio placentae
- c) Cervical polyp
- d) Placental infarction

Q72. A 33-year-old woman at 34 weeks gestation has stable placenta previa. What is the next step in management?

- a) Immediate hospitalization and emergency cesarean section
- b) Expectant management with periodic TVS and McAfee-Johnson regimen



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- c) Magnesium sulfate and immediate delivery
- d) Monthly abdominal ultrasound only

Q73. Which of the following are the most common causes of antepartum hemorrhage?

- 1. Placenta previa
 - 2. Placental abruption
 - 3. Uterine rupture
 - 4. Vasa previa
- a) 1 and 2
 - b) 2 and 3
 - c) 3 and 4
 - d) 1 and 4

Q74. A 30-year-old woman with severe dysmenorrhea, dyspareunia, infertility, dense pelvic adhesions, and ovaries with ovarian cyst measuring 7x6 cm and 7x7 cm each stucked together on laparoscopy is diagnosed with endometriosis. Next step in management?

- a) LUNA
- b) Gestrinone
- c) Cystectomy and adhesiolysis
- d) Hysterectomy and oophorectomy

Q75. A 25-year-old woman with PMOS is on ovulation induction with Clomiphene citrate but fails to conceive after 6 cycles. She is overweight (BMI 32), has insulin resistance, and ultrasound shows multiple small follicles with thickened stroma. Which of the following is the next best step in her management?

- a) Switch to Letrozole
- b) Add Metformin and continue induction
- c) Laparoscopic ovarian drilling
- d) IVF with ICSI

Q76. A 28-year-old woman presents with fatigue, low-grade fever, pain and swelling of multiple small joints of the hands, recurrent painless oral ulcers, and a facial rash that becomes more prominent after sun exposure. Urinalysis reveals proteinuria.

Which of the following autoantibodies is the most sensitive for the diagnosis of SLE?

- a) Anti-Smith (Anti-Sm) antibody
- b) Anti-double-stranded DNA (Anti-dsDNA) antibody
- c) Antinuclear antibody (ANA)
- d) Anti-histone antibody



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q77. A 52-year-old woman presents with a 10-year history of episodic blanching and bluish discoloration of her fingers on exposure to cold. Over the years, she has developed progressive tightening of the skin over her fingers and face, difficulty swallowing solid foods, and multiple telangiectasias over her cheeks and hands. Examination reveals sclerodactyly and calcified nodules over the fingertips. Laboratory investigations show a positive anti-centromere antibody. Two years later, she develops gradually worsening exertional dyspnea, fatigue, and occasional presyncope. Chest auscultation is unremarkable. Echocardiography demonstrates elevated pulmonary artery pressures with normal left ventricular function.

Which of the following is the most likely complication responsible for her current symptoms?

- a) Reflux esophagitis
- b) Scleroderma renal crisis
- c) Pulmonary arterial hypertension
- d) Autoimmune myocarditis

Q78. A 45-year-old obese male with metabolic syndrome presents with recurrent attacks of severe pain in the great toe. He has had three episodes in the last year. Uric acid is 9.2 mg/dL. He takes thiazide diuretics for his hypertension. What is the best long-term management?

- a) Stop thiazide and start losartan
- b) Start NSAIDs for prophylaxis
- c) Start allopurinol now
- d) Give intra-articular steroids

Q79. A 19-year-old woman is evaluated for intermittent jaundice since childhood. She denies abdominal pain, pruritus, pale stools, or dark urine. Physical examination is unremarkable except for mild scleral icterus. Laboratory investigations reveal:

Total bilirubin: 3.8 mg/dL

Predominantly conjugated hyperbilirubinemia

Normal AST, ALT, and ALP

Negative viral hepatitis serology

Liver biopsy shows no pigment deposition, and the liver appears grossly normal.

Which of the following genetic defects is most likely responsible for this condition?

- a) ABCC2 (MRP2) mutation
- b) UGT1A1 mutation
- c) OATP mutations
- d) ATP7B mutation

Q80. A 45-year-old man with no symptoms undergoes a routine medical check. He is a migrant worker from an HBV-endemic region.

Serology:**

HBsAg: Positive for >6 months

Anti-HBc IgG: Positive

HBeAg: Positive

HBV DNA: High

Anti-HBs: Negative



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

LFTs: ALT mildly elevated.

What is the correct interpretation?

- a) Past infection with immunity
- b) Vaccination-induced immunity
- c) Chronic Hepatitis B with high viral replication
- d) Acute resolving Hepatitis B

Q81. A 45-year-old woman presents with progressive fatigue, intense pruritus, and mild jaundice for the past 8 months. Laboratory investigations reveal markedly elevated ALP and GGT with a cholestatic pattern of liver injury. Liver biopsy shows chronic destruction of small intrahepatic bile ducts with portal inflammation.

Which of the following antibodies is most likely to be positive in this patient?

- a) Anti-smooth muscle antibody (ASMA)
- b) Anti-liver kidney microsomal (Anti-LKM-1) antibody
- c) Anti-mitochondrial antibody (AMA)
- d) Anti-neutrophil cytoplasmic antibody (p-ANCA)

Q82. A 16-year-old boy is brought to the clinic because of declining school performance, tremors, and difficulty writing over the past year. His parents also report recent behavioral changes and irritability. On examination, he has mild hepatomegaly and a resting tremor. Slit-lamp examination reveals golden-brown rings at the corneal periphery. Laboratory investigations show:

Decreased serum ceruloplasmin

Increased urinary copper excretion

Elevated hepatic copper content

Mutation in which of the following genes is most likely responsible for this condition?

- a) HFE
- b) ATP7B
- c) ATP7A
- d) ABCC2

Q83. A 58-year-old man with a 15-year history of type 2 diabetes mellitus presents with progressive fatigue and generalized weakness for the past few months. He is on insulin and antihypertensive medications.

Physical examination is unremarkable except for mild peripheral neuropathy.

Laboratory investigations reveal:

Serum bicarbonate (HCO_3^-): 18 mEq/L

Serum potassium: 6.1 mEq/L

Arterial blood gas: normal anion gap metabolic acidosis

Urine pH: 5.0

Serum creatinine: mildly elevated

Further evaluation suggests hyporeninemic hypoaldosteronism. Which of the following is the most likely diagnosis?

- a) Type I (Distal) Renal Tubular Acidosis
- b) Type II (Proximal) Renal Tubular Acidosis
- c) Type IV Renal Tubular Acidosis
- d) Gitelman syndrome



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q84. A 68-year-old man with long-standing hypertension presents with poorly controlled blood pressure despite multiple antihypertensive medications. On examination, an abdominal bruit is heard in the periumbilical region. He is started on an ACE inhibitor for better blood pressure control.

One week later, laboratory investigations show:

Serum creatinine increased from 1.1 mg/dL to 2.4 mg/dL

Serum potassium: mildly elevated

No evidence of dehydration or urinary obstruction

The physician suspects a renovascular cause for his hypertension. Which of the following is the most likely underlying diagnosis?

- a) Unilateral renal artery stenosis
- b) Bilateral renal artery stenosis
- c) Renal vein thrombosis
- d) Fibromuscular dysplasia

Q85. A 48-year-old man presents with progressive swelling of the legs and frothy urine for the past 3 months. On examination, he has bilateral pitting pedal edema. Laboratory investigations reveal:

Urine protein: 4+

24-hour urinary protein: 7 g/day

Serum albumin: 2.2 g/dL

Serum cholesterol: 340 mg/dL

Serum creatinine: Normal

A renal biopsy is performed. Light microscopy shows diffuse thickening of the glomerular capillary wall, and silver stain demonstrates "spike and dome" appearance. Immunofluorescence reveals granular IgG and C3 deposition along the glomerular basement membrane. Which of the following antibodies is most commonly associated with primary membranous nephropathy?

- a) Anti-glomerular basement membrane antibody
- b) Anti-phospholipase A2 receptor (PLA2R) antibody
- c) Anti-double-stranded DNA antibody
- d) Anti-neutrophil cytoplasmic antibody (ANCA)

Q86. A 45-year-old woman presents with progressive fatigue, weight loss despite increased appetite, palpitations, and heat intolerance for the past 4 months. She also reports bone pain and recurrent renal stones. On examination, she has a resting tachycardia and mild proximal muscle weakness.

Laboratory investigations reveal:

Serum calcium: 11.2 mg/dL (high)

Serum phosphate: low

Serum PTH: elevated

Serum alkaline phosphatase: elevated

Ultrasound of the neck shows a solitary parathyroid nodule.

Which of the following is the most likely diagnosis?

- a) Secondary hyperparathyroidism
- b) Tertiary hyperparathyroidism
- c) Primary hyperparathyroidism
- d) Pseudohypoparathyroidism



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q87. A 38-year-old woman presents with progressive weight gain, rounding of the face, easy bruising, and weakness of proximal muscles for the past 8 months. She also reports irregular menstrual cycles and new-onset hypertension. On examination, she has purple striae over the abdomen, central obesity, and thin fragile skin.

Laboratory investigations reveal:

Fasting blood glucose: 180 mg/dL

Serum cortisol: elevated

Plasma ACTH: elevated

Low-dose dexamethasone test shows no suppression of cortisol levels. Which of the following is the most likely diagnosis?

- a) Adrenal adenoma (ACTH-independent Cushing syndrome)
- b) Cushing disease (pituitary adenoma)
- c) Ectopic ACTH production
- d) Exogenous steroid intake

Q88. A 72-year-old man with a history of type 2 diabetes mellitus and poor medication adherence is brought to the emergency department with progressive confusion, extreme thirst, and decreased urine output for the past 2 days. His family reports no history of vomiting or abdominal pain.

On examination:

Patient is severely dehydrated

Blood pressure: 90/60 mmHg

Heart rate: 118/min

Laboratory investigations reveal:

Random blood glucose: 780 mg/dL

Serum osmolality: high

Urine ketones: negative

Arterial blood gas: near normal pH

Which of the following is the most likely diagnosis?

- a) Diabetic ketoacidosis
- b) Hyperosmolar hyperglycemic state
- c) Lactic acidosis
- d) Acute hypoglycemia

Q89. A 5-day-old neonate develops severe jaundice and poor feeding. Laboratory investigations reveal marked unconjugated hyperbilirubinemia. Despite intensive phototherapy, bilirubin levels remain extremely high, and the infant develops signs of kernicterus. There is no evidence of hemolysis.

Which of the following is the most likely underlying defect?

- a) Partial deficiency of UGT1A1 enzyme
- b) Complete absence of UGT1A1 enzyme
- c) Defective canalicular bilirubin transporter (MRP2)
- d) Defective OATP1B1/OATP1B3 transporters



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q90. A 63-year-old man with a 20-year history of chronic hepatitis B presents with unintentional weight loss, abdominal discomfort, and new onset abdominal swelling. Exam reveals shifting dullness and mild jaundice.

Laboratory results:

AST: 145 U/L

ALT: 110 U/L

ALP: mildly elevated

AFP (alpha-fetoprotein): markedly elevated

Abdominal ultrasound shows a heterogeneous mass in the right lobe of the liver with arterial enhancement and rapid venous washout on CT.

What is the most likely diagnosis?

- a) Cholangiocarcinoma
- b) Hepatocellular carcinoma
- c) Liver metastasis
- d) Focal nodular hyperplasia

Q91. A 48-year-old man with a history of chronic alcohol consumption presents with progressive jaundice, abdominal pain, fever, and tender hepatomegaly. Laboratory investigations reveal:

Serum bilirubin: 18 mg/dL

Prothrombin time: 24 seconds

Control prothrombin time: 12 seconds

AST: 180 U/L

ALT: 70 U/L

The hepatologist calculates a Maddrey Discriminant Function (DF) score to assess disease severity and determine the need for specific therapy.

Which of the following is the most appropriate interpretation if the calculated DF is >32 ?

- a) Mild alcoholic hepatitis; supportive treatment only
- b) Severe alcoholic hepatitis; corticosteroid therapy should be considered
- c) Acute liver failure requiring immediate transplantation in all cases
- d) Chronic cirrhosis with good prognosis

Q92. A 34-year-old woman presents with progressive abdominal distension, right upper quadrant pain, and swelling of both legs for the past 2 weeks. She has a history of recurrent headaches and facial plethora. On examination, she has ascites, tender hepatomegaly, and mild jaundice. Laboratory investigations reveal elevated liver enzymes. Doppler ultrasonography demonstrates absence of flow in the hepatic veins.

Which of the following is the most likely diagnosis?

- a) Portal vein thrombosis
- b) Budd–Chiari syndrome
- c) Primary sclerosing cholangitis
- d) Acute viral hepatitis

Q93. A 58-year-old man with a 15-year history of type 2 diabetes mellitus and hypertension presents with progressive fatigue, loss of appetite, and generalized weakness. He reports nocturia and swelling of both feet for the past several months. On examination, he has bilateral pedal edema and pallor.



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Laboratory investigations reveal:

Serum creatinine: 3.8 mg/dL

Estimated GFR: 22 mL/min/1.73 m²

Serum bicarbonate (HCO₃⁻): 16 mEq/L

Serum potassium: 5.6 mEq/L

Urinalysis: proteinuria (++)

Which of the following is the most likely cause of the metabolic acidosis seen in this patient?

- a) Excessive bicarbonate loss from the gastrointestinal tract
- b) Reduced renal excretion of hydrogen ions and impaired ammonium generation
- c) Increased production of lactic acid by skeletal muscle
- d) Hyperventilation causing respiratory alkalosis

Q94. A 19-year-old college student presents with episodes of visible blood in urine occurring repeatedly over the past year. He reports that each episode develops within 1–2 days of an upper respiratory tract infection and resolves spontaneously after a few days. There is no history of edema, hypertension, or renal stones.

Laboratory investigations reveal:

Urinalysis: Hematuria with red blood cell casts

Mild proteinuria

Serum complement (C3): Normal

A renal biopsy is performed. Which of the following is the most likely diagnosis?

- a) Post-streptococcal glomerulonephritis
- b) IgA nephropathy (Berger disease)
- c) Membranous nephropathy
- d) Minimal change disease

Q95. A 36-year-old woman presents with progressive fatigue, weight gain, and intolerance to cold for the past 6 months. She also complains of constipation and hair thinning. There is no history of thyroid surgery or radiation exposure.

On examination, she has dry skin, bradycardia, and a diffusely enlarged, non-tender thyroid gland.

Laboratory investigations reveal:

TSH: Elevated

Free T4: Decreased

Which of the following antibodies is most commonly associated with this condition?

- a) Anti-TSH receptor antibody
- b) Anti-thyroid peroxidase (anti-TPO) antibody
- c) Anti-thyroglobulin receptor antibody
- d) Anti-centromere antibody

Q96. 42-year-old man presents with long-standing hypertension that is poorly controlled despite multiple medications. He also complains of intermittent muscle weakness and cramps.

On examination, his blood pressure is 170/100 mmHg.

Laboratory investigations reveal: Elevated plasma aldosterone–renin ratio

Which of the following is the investigation of choice to confirm the diagnosis?

- a) Plasma aldosterone–renin ratio



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- b) CT scan of adrenal glands
- c) Saline infusion test
- d) 24-hour urinary metanephrines

Q97. A 38-year-old woman presents with weight gain, moon face, proximal muscle weakness, and purple abdominal striae. On examination, she is hypertensive. Screening tests suggest hypercortisolism.



Which of the following is the confirmatory test for this condition?

- a) Overnight low-dose dexamethasone suppression test
- b) 24-hour urinary free cortisol
- c) Late-night salivary cortisol
- d) Low-dose dexamethasone suppression test (48-hour)

Q98. A 30 year old healthy women presents with headache neck stiffness photophobia and vomiting. Cerebrospinal fluid analysis shows WBC 700 (98% lymphocytes), Proteins 1.5 % Glucose 85 mg/dl, HSV-2 antibodies were detected .This type of meningitis is associated with :

- a) Recurrence after complete recovery is extremely rare
- b) Is a complication of primary genital herpes
- c) Has mortality rate of 60%
- d) Seizure is not a feature

Q99. 60 year old male presented to you with the c/o sharp central chest pain from last 6 hrs associated with increase during inspiration. His previous records s/o AAMI 10 days back for which he under went PTCA to L.A.D territory ECG is s/o Increase in PR interval with ST elevation . What is the appropriate treatment

- a) Ibuprofen
- b) Thrombolysis
- c) Aspirin
- d) Angiography

Q100. A 26-year-old school teacher had been diagnosed with hepatitis B infection 2 years earlier. She was now worried about whether the infection was still active. Which of the following test results is most sensitive at confirming continued viral replication?

- a) Anti-HBe
- b) Anti-HBs
- c) HBeAg
- d) HBV DNA



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- Q101. A patient presents with malar rash, photosensitivity, oral ulcers, arthritis, and signs of nephritic syndrome. Upon examination of his kidney, there appears to be presence of wireloop lesions. Test samples reveal antibodies against dsDNA. What is the pathogenic mechanism of the disease?
- Immune complex mediated
 - Infection
 - Tumor
 - None of the above
- Q102. 27 years old female compliance of severe headaches. She has experiencing this headache since her teenage, but over the past few years this headache has become more frequent and now occurs on weekly basis. The headache is unilateral throbbing and associated with nausea , photophobia and phonophobia. They last up to 24 to 48 hours at a time. She is taking O.C.P. Neurological examination is normal. MRI head is also normal. Which of the following is not advised for prophylaxis for this patients headache .
- Sumatriptan
 - Topiramate
 - Propranolol
 - Amitriptyline
- Q103. ECG finding in a patient of acute m.i that makes a candidate suitable for reperfusion therapy is?
- St elevation of atleast 1mm in two contiguous precordial leads
 - St elevation of at least 2 mm in two contiguous precordial leads
 - St elevation of at least 1 mm in three contiguous precordial leads
 - St- segment elevation of at least 2mm in three contiguous precordial leads
- Q104. Which of the following artery supplying dominant hemisphere is occluded in wernicks aphasia without weakness?
- Inferior division of m.c.a
 - Stem of mca
 - Proximal superior division of m.c.a
 - Penetrating lenticulostriate arteries of m.c.a
- Q105. A 24 year old female presents with abdominal pain, hematuria and arthralgia. On examination, palpable purpuric rash was seen on trunk, limbs and buttocks. Coagulation tests were normal. What is the most probable diagnosis?
- Hemolytic uremic syndrome
 - Henoch-Schönlein purpura
 - Meningococemia
 - Thrombotic thrombocytopenic purpura
- Q106. Which of these statements is incorrect about growth in children?
- Cephalocaudal sequence
 - IGF-1 and IGF-2 important for intrauterine growth
 - Mid-arm circumference used for growth assessment between birth to 5 years
 - Adolescent growth spurt is seen



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q107. Which of these milestones in the child is last to develop?

- a) Hopping
- b) Riding a tricycle
- c) Speaking sentences
- d) Complete bladder control

Q108. A newborn is delivered after a LSCS and has a weak cry at birth. The baby is sluggish with some respiratory distress and intercostal retractions. His APGAR score at 5 minutes is 4. The baby will be categorized as:

- a) Normal
- b) Mildly depressed
- c) Moderately depressed
- d) Severely depressed

Q109. A child is able to climb steps maturely, both while going upstairs and downstairs. He can also ride a tricycle and can identify colors. Which of these milestones the child will not be able to perform?

- a) Making tower of 10 cubes
- b) Develop right and left handedness
- c) Copy a triangle
- d) Identify gender

Q110. A 29-week preterm newborn has two episodes of uprolling and right deviation of eyeballs while in the NICU. Cranial ultrasound reveals an intracranial hemorrhage. Which of these is likely to be the most common intracranial region for high propensity of hemorrhage in this baby?

- a) Caudate nucleus
- b) Germinal matrix
- c) Brainstem
- d) Peripheral cerebral cortex

Q111. An 8-year-old boy is brought to the pediatric OPD for evaluation of short stature. His parents are short, father and mother height being 162 cm and 152 cm respectively. What is the approximate mid-parental height this child is likely to achieve?

- a) 156 cm
- b) 158.5 cm
- c) 160 cm
- d) 163.5 cm



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q112. Which of the following drug classes should be started as front-line controller treatment in a boy with frequent episodes of acute asthma exacerbations?

- a) Inhaled corticosteroids
- b) Leukotriene receptor antagonists
- c) Long-acting beta agonists
- d) Short-acting beta-agonists

Q113. In a 36-week preterm baby, abdominal radiograph shows absence of spinal segments below a hypoplastic L1 segment. A diagnosis of caudal regression syndrome was made. There is a strong association of this condition with:



- a) Maternal intake of antiepileptic drugs
- b) Maternal epilepsy
- c) Maternal obesity
- d) Maternal diabetes

Q114. All of the following are common features of preterm babies except:

- a) Long nails
- b) Plenty of lanugo
- c) Absent sole creases
- d) Thin red skin

Q115. A child with cyanosis and breathing difficulty is brought to the emergency. There is a history of lithium intake during pregnancy by the mother. The child is diagnosed on echocardiogram with Ebstein anomaly. Which of the following cardiac chambers are likely to be hypoplastic in this condition?

- a) RA
- b) RV
- c) LA
- d) LV

Q116. Which congenital heart disease is most likely in a child diagnosed with Noonan syndrome?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- a) ASD
- b) VSD
- c) Subvalvular pulmonary stenosis
- d) Supravalvular aortic stenosis

Q117. A child with MAC of 110 mm with bilateral pedal edema and poorly taking feeds should be categorized as:

- a) Chronic malnutrition
- b) Moderate malnutrition
- c) Uncomplicated severe acute malnutrition
- d) Complicated severe acute malnutrition

Q118. Which of these anthropometric markers is believed to be the best for diagnosing wasting in children?

- a) Weight of age
- b) Weight for height
- c) Height for age
- d) MAC

Q119. A child with a rapidly enlarging head size from birth and ventriculomegaly shown on brain imaging is diagnosed with obstructive hydrocephalus. Which of these is most likely cause?

- a) Measles
- b) Congenital
- c) CMV
- d) Meningitis

Q120. Which of these is the drug of choice for treatment of nephrotic syndrome in a child not responding to steroid therapy?

- a) MMF
- b) Levamisole
- c) Cyclophosphamide
- d) Tacrolimus

Q121. De Quervain's disease classically affects:

- a) Extensor pollicis longus and abductor pollicis longus
- b) Extensor pollicis longus and abductor pollicis brevis
- c) Extensor pollicis brevis and abductor pollicis brevis
- d) Extensor pollicis brevis and abductor pollicis longus



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q122. A 35-year-old woman presents with pain and swelling at the posterior aspect of the heel. The pain worsens while wearing closed-back shoes. On examination, there is a prominent bony swelling at the posterosuperior aspect of the calcaneus with tenderness over the insertion of the Achilles tendon. What is the most likely diagnosis?

- a) Sever disease
- b) Haglund deformity
- c) Plantar fasciitis
- d) Calcaneal stress fracture

Q123. A 12-year-old boy falls on an outstretched hand and presents with pain and deformity of the forearm. X-ray reveals a fracture of the proximal third of the ulna associated with anterior dislocation of the radial head at the elbow. What is the most likely diagnosis?



- a) Galeazzi fracture-dislocation
- b) Monteggia fracture-dislocation
- c) Essex-Lopresti injury
- d) Colles fracture

Q124. A 16-year-old boy presents with progressively increasing pain and swelling around the knee for 3 months. The pain is worse at night and is not relieved by rest. X-ray shows a metaphyseal lesion in the distal femur with a “sunburst” appearance and Codman’s triangle. What is the most likely diagnosis?

- a) Osteoid osteoma
- b) Osteosarcoma
- c) Ewing sarcoma
- d) Giant cell tumor



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q125. A 28-year-old man sustains a fracture neck of femur following a road traffic accident. Six months later, he develops persistent hip pain and limping. X-ray shows collapse of the femoral head. Which of the following complications is most likely responsible for this finding?

- a) Malunion
- b) Non-union
- c) Avascular necrosis
- d) Myositis ossificans

Q126. A man takes peanuts and develops, tongue swelling, neck swelling, stridor, hoarseness of voice. What is the probable diagnosis?



- a) Angioneurotic edema
- b) Foreign body in bronchus
- c) Parapharyngeal abscess
- d) Foreign body in larynx

Q127. All are seen in Behcet's syndrome except:

- a) Genital ulcers
- b) Uveitis
- c) Erythema nodosum
- d) Pyoderma gangrenosum

Q128. All are true regarding hereditary angio-edema except:

- a) Bradykinin level increase during attack
- b) Enzyme involved in C1 esterase inhibitor
- c) C1 inhibitor targets Hageman factor
- d) Complement C4 and C2 decrease

Q129. A patient gives a history of recurrent oral ulcers. The ulcers are small with a yellow floor surrounded by an erythematous halo on the lips. He also has multiple, tender nodules on the shin. He most likely has:



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026



- a) Behcet's syndrome
- b) Herpes labialis
- c) Fixed drug eruption
- d) Pemphigus vulgaris

Q130. Acute febrile neutrophilic dermatosis is:

- a) Kasabach-Merit syndromé
- b) Sweet's syndrome
- c) Behcet's syndrome
- d) Mucha-Habermann disease

Q131. A middle aged person reported to Psychiatric OPD with the complaints of fear of leaving home, fear of travelling alone and fear of being in a crowd. He develops marked anxiety with palpitations and sweating if he is in these situations. He often avoids public transport to go his place of work. His likely diagnosis is?

- a) GAD
- b) Schizophrenia
- c) Agoraphobia
- d) OCD

Q132. A 57 year old man, who had been hospitalized for an acute pneumonia 3 days previously, starts to shout in the bed, stating that, "someone has come to room to kill him." He then gets out of bed and begins pulling out his IV line. On examination, he is not oriented to time/ place. Which of the following diagnosis best fits this patient's clinical picture?

- a) Dementia
- b) Schizophrenia
- c) Delirium
- d) Hysteria

Q133. Expand IDEAS :

- a) International disability evaluation and assessment Scale
- b) Indian disability evaluation and assessment Scale
- c) Indian disorder evaluation and assessment Scale
- d) None of above



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q134. mhGAP programme includes all of the following disorders except?

- a) Depression
- b) Substance use disorder
- c) Panic disorder
- d) Epilepsy

Q135. After use of some drug, a person develops episodes of rage in which he runs about and indiscriminately injures a person who is encountered in way. He is probably addict of?

- a) Cannabis
- b) Cocaine
- c) Alcohol
- d) Opium

Q136. Identify the false statement about the Radiation exposure?

- a) Radiation exposure in mammography is 10 to 40 times CXR
- b) Radiation exposure of Barium enema is more than PET Scan alone.
- c) Radiation exposure of CT Chest is 1000 times CXR
- d) CT Head & IVU have similar radiation exposure

Q137. The least RADIORESISTANT phase of the cell cycle is:

- a) G2-M phase
- b) S phase
- c) G1 phase
- d) M phase

Q138. Identify the marked structure?



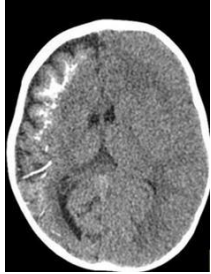
- a) Pituitary Gland
- b) Rostrum, Corpus Callosum
- c) Splenium, Corpus Callosum
- d) Pineal Gland

Q139. Identify the abnormality shown in the given below image of a child presenting with familial neuro-cutaneous synrome?



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026



- a) NF-1
- b) NF-2
- c) Sturge Weber Syndrome
- d) Tuberous Sclerosis

Q140. In a non-palpable mammographic lesion classified as BI-RADS 3, the recommended management is:

- a) Stereotactic core biopsy
- b) Surgical excision biopsy
- c) Short-interval mammographic follow-up at 6 months
- d) Annual mammographic follow-up only

Q141. A 30-year-old male presents with an incidental finding on spinal X-ray — a vertebral body with a 'jail bar' / corduroy appearance. What is the diagnosis?

- a) Vertebral metastasis
- b) Multiple myeloma
- c) Vertebral haemangioma
- d) Eosinophilic granuloma

Q142. Identify the investigation shown below?





MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

- a) OPG
- b) Intra-oral Dental Radiography
- c) Extra-oral Dental Radiography
- d) None of the above

Q143. Identify the Investigation and the marked structure?



- a) MCU and Normal tapering at Bulbo-membranous urethra junction.
- b) RGU and Urethral stricture at Membranous Urethra
- c) MCU and Urethral stricture at Membranous Urethra
- d) RGU and Normal tapering at Bulbo-membranous urethra junction.

Q144. Identify the investigation and its primary application?



- a) Trans-esophageal Echocardiography & Left atrial Thrombus
- b) Trans-esophageal USG & Loco-regional staging of G.I. malignancy
- c) Trans-esophageal USG and grading severity of enteritis
- d) Trans-esophageal Echocardiography & atrial Fibrillation

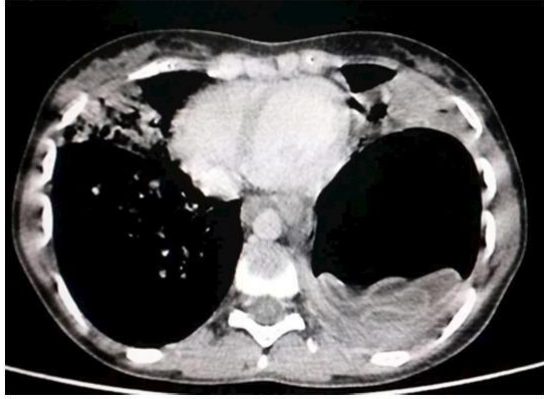
Q145. Identify the abnormality shown on the CT Chest of a patient presenting with expectoration of copious cough & urticaria :





MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026



- a) Cysticercosis
- b) Echinococcosis
- c) Tuberculosis
- d) Pneumoconiosis

Q146. A 45-year-old man undergoing surgery under general anesthesia is intubated, paralysed with a neuromuscular blocking agent, and maintained on intermittent positive pressure ventilation (IPPV). Which of the following Mapleson breathing circuits is most appropriate?

- a) Mapleson A
- b) Mapleson B
- c) Mapleson C
- d) Mapleson D

Q147. A 65-year-old man with a history of coronary artery disease underwent drug-eluting stent placement 1 year ago. He is currently taking aspirin and clopidogrel. He is now posted for elective cholecystectomy, and the surgeon is concerned about perioperative bleeding. What is the most appropriate management of clopidogrel before surgery?

- a) Continue clopidogrel perioperatively
- b) Stop clopidogrel 24 hours before surgery
- c) Stop clopidogrel 5 days before surgery
- d) Postpone surgery

Q148. A 35-year-old man is taken for emergency laparotomy under general anesthesia. Rapid-sequence induction is performed with a high dose of a steroidal nondepolarizing neuromuscular blocker. Immediately after intubation, the procedure is cancelled due to an intraoperative concern, and the patient must be awakened. Neuromuscular monitoring shows no response to train-of-four stimulation. Which of the following is the most appropriate management for rapid reversal of neuromuscular blockade?

- a) Neostigmine + glycopyrrolate
- b) Sugammadex 2 mg/kg IV
- c) Sugammadex 16 mg/kg IV
- d) Flumazenil IV



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

Q149. While preparing the anesthesia workstation, a gas cylinder is mounted onto the yoke assembly. The safety pin index configuration on the cylinder corresponds to positions 3 and 5. Which gas does this cylinder most likely contain?

- a) Medical air
- b) Entonox
- c) Oxygen (O₂)
- d) Nitrous oxide (N₂O)

Q150. A 28-year-old woman is evaluated in the pre-anesthetic clinic before elective surgery. She is seated upright and asked to open her mouth fully and protrude her tongue without phonation. According to the Modified Mallampati classification, the airway shown corresponds to:



- a) Grade I
- b) Grade II
- c) Grade III
- d) Grade IV

ANSWERS

1	d
2	b
3	d
4	b
5	a
6	a
7	a
8	c
9	a
10	c
11	b
12	d
13	c
14	a

15	a
16	b
17	b
18	c
19	b
20	c
21	b
22	b
23	b
24	c
25	b
26	b
27	b
28	c

29	b
30	c
31	c
32	a
33	b
34	b
35	b
36	b
37	c
38	a
39	b
40	b
41	c
42	b

43	b
44	a
45	c
46	b
47	c
48	c
49	b
50	c
51	d
52	d
53	b
54	a
55	c
56	a

57	c
58	d
59	c
60	a
61	d
62	c
63	c
64	a
65	d
66	c
67	a
68	d
69	b
70	d



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST - 6 PAPER - 2 DATED ON 16th JUNE 2026

71	b
72	b
73	a
74	c
75	c
76	c
77	c
78	a
79	c
80	c
81	c
82	b
83	c
84	b
85	b
86	c

87	b
88	b
89	b
90	b
91	b
92	b
93	b
94	b
95	b
96	c
97	d
98	b
99	a
100	d
101	a
102	c

103	b
104	a
105	b
106	c
107	d
108	c
109	c
110	b
111	d
112	a
113	d
114	a
115	b
116	c
117	d
118	b

119	b
120	d
121	d
122	b
123	b
124	b
125	c
126	a
127	d
128	a
129	a
130	a
131	c
132	c
133	b
134	c

135	a
136	c
137	a
138	b
139	c
140	c
141	c
142	a
143	d
144	b
145	b
146	d
147	c
148	c
149	d
150	c